



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : دعاء خيرى عبدالحليم محمد

العنوان : حلوان

الحي / المدينة : حلوان

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 24-09-2026

رقم البطاقة : 27701010127262 رقم التليفون : 01155622231

العمر (تاريخ الميلاد) : 01-01-1977 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: دعاء خيرى عبدالحليم محمد National ID: 27701010127262	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢١/٠٦ ٢٧٧٠١٠١٠١٢٧٢٦٢
حاصلة على بكالوريوس عربية
رياض الطفل
التي مسلمة
خمس ابراهيم خفي حسن
البيانات مدارية خفي ٢٠٢٨/٠٦/٠٧



جيهة شؤون كذا في مصر العربية
بطاقة تحقيق الشخصية

دعاء
خيري عبد الحكيم محمد
١٩ بلوك ١ محفورة ١٠٠ المشروع الامريكي
حوران - القاهرة

١٩٨٧/٠٦

٢ ٧٧ ٠١ ٠١ ٠١ ٢٧٢ ٦٢

IX5227230

Psychiatric

Patient Name : الأملأه / دواء خيري عبدالحليم	Request No. : 13003559
Patient No. : 27701010127262	Request Date : 20/08/2025
Path No. : 2593/25	Verification Date : 07/09/2025
Sex : Female	Financial Category : فاقة الدولة - جومفاه روسي
Age : 48 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Up-Front.

Natura Specimen: Right simple mastectomy & sentinel lymph nodes biopsy.

Gross Examination:

Right simple mastectomy specimen measured 18x15.5x4 cm, with covering skin measured 17x8.5 cm. Nipple & Areola looked unremarkable grossly. Serialing showed two masses measured 2.7x2.5x1.7 cm & 6x3.5x2 cm, located 1.5 cm apart and were 1 cm away from the least deep margin. Also received in the same container two unremarkable skin strips 19x1 cm & 10x1 cm. Sentinel lymph nodes: Fibrofatty tissue pieces collectively measured 3x3 cm, entangling 3 lymph nodes, the largest was 2x1.5x1 cm.

15 Blocks:

3- 1st Mass., 4- Area inbetween., 5:12- Other mass., 13- Deep margin., 14- N&A with skin strips., 15- SLNs.

Gross was done by; Dr. Mennan Hossam.

Frozen Results:

Positive metastatic deposits in one out of three sentinel lymph nodes (1/3).

Microscopic Examination:

Histological Type: Bifocal invasive lobular carcinoma.
Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
Ductal carcinoma in situ: Absent.
Lobular carcinoma insitu: Present.
Angioinvasion: Present.
Perineural invasion: Present.
Nipple & areola: Free.
Deep margin: Free.

Verified by :

Dr. Duaa Mohamed

Printing Date : 09/09/2025 09:49:21

Page 1 of 3

Printing By : marwa mohamed youss



غرفا رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / دعاء خيري عبدالجليم
Patient No. : 27701010127262
Path No. : 2593/25
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13003559
Request Date : 20/08/2025
Verification Date : 07/09/2025
Financial Category : نفقة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hagazy

Pathology Report

Sentinel lymph nodes: Positive metastatic deposits in one out of three sentinel lymph nodes (1/3) with no extra-nodal extension (The metastatic focus was clearly identified in one lymph node during intraoperative frozen section examination. The corresponding paraffin embedded sections did not reveal this focus due to levels variation during preparation, and the frozen section diagnosis is considered valid).

Immunophenotyping:

Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; E-Cadherin.
Ecadherin: Negative in tumor cells.

Diagnosis:

Right breast cancer (Up-front), Right simple mastectomy & sentinel lymph nodes biopsy.
Bifocal invasive lobular carcinoma, Grade 2, with lobular carcinoma insitu (LCIS).
Ductal carcinoma in situ: Absent.
Deep margin: Free.
Nipple & areola: Free.
Sentinel lymph nodes: Positive metastatic deposits in one out of three sentinel lymph nodes (1/3) with no extra-nodal extension.
Stage: pT3(m), pN1a(sln).

Comment:

The metastatic focus was clearly identified in one lymph node during intraoperative frozen section examination. The corresponding paraffin embedded sections did not reveal this focus due to levels variation during preparation, and the frozen section diagnosis is considered valid.

Signature

Verified by :
Dr. Duaa Mohamed

Printing Date : 09/09/2025 09:49:21

Page 2 of 3

Printing By : marwa mohamed youss



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Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mistr El Qadema - Cairo, Egypt

Patient Name : الأمل، / دعاء فخرى عبدالحكيم
Patient No. : 27701010127262
Path No. : 2188/25
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13003993
Request Date : 02/07/2025
Verification Date : 07/07/2025
Financial Category : ثلثة ابرولة - اوزام دار السلام
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data
Right breast mass

Nature of specimen
True cut biopsy, collected on 1/7/2025

Gross examination

Multiple grayish white tissue cores up to 1.5 cm, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- Histological type: Mixed Invasive duct & lobular carcinoma
- Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade 2
- DCIS: Focal, low grade (solid pattern)
- LCIS: Focal
- Angiolymphatic invasion: Couldn't be detected in the received biopsy
- Perineural invasion: No
- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
- PR: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
- HER2: No membranous reaction in of tumor cells (score 0)
- Ki67 20%

Diagnosis

Verified by :
Dr. Hadea Hassan

Printing Date: 14/07/2025 09:23:39

Page 1 of 2

Printed By: Mrs. Mayada Hegazy

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Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Msr El Qadima - Cairo, Egypt



Patient Name: Doaa Khairi Abdel Haleem Mohamed	Date : 19/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 48 Year(s)
Medical ID: 2022/2025	Pat. ID: 27596

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Chest:-

- Right breast small speculated lesion for sonomammographic assessment.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavity lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen & Pelvis:

- The liver is of average size with homogenous CT texture showing small non-enhancing lesion at segment VIII measuring 7 mm likely cyst.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.

د. محمد حسن
 19/07/2025
 10:27:22

Patient Name: Doaa Khairi Abdel Haleem Mohamed	
Exam: CT CAP+C (CT Chest Abd. Pelvis)	Date: 19/07/2025
Medical ID: 2022/2025	Age: 48 Year(s)
	Pat. ID: 27596

Normal appearance of the other pelvic organs.
 No abdominal or pelvic masses or collections.

Scanned skeleton: no definite worrisome lytic or sclerotic focal lesions.

- Opinion:**
- Right breast small speculated lesion for sonomammographic assessment.
 - Small non-enhancing lesion at segment VIII likely cyst.
 - No evidence of pulmonary, hepatic or osseous metastatic deposits.

Signature Dr. Mohamed Hassan
 23/07/2025 04:23

Patient ID: 86472	Patient Name English: Dosa Khalry Abd Alhaleem
Dr. F. YAGHI	2025/07/30 09:51:47 AM :Report Date

Contrast enhanced Spectral Mammography
CLINICAL SUMMARY: Right breast pathologically proven mixed invasive ductal and lobular carcinoma.

TECHNIQUE: CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: b (scattered fibro-glandular densities)

RIGHT BREAST:

- Low energy (Non contrast) images: show upper outer quadrant scattered intercommunicating irregular equal density masses and nodules.
- Recombined (Post contrast) images: show corresponding upper outer quadrant and upper central (extending from 10 o'clock to 12:30 o'clock, based on targeted US examination) multiple heterogeneously enhancing irregular masses and nodules, associated with regional and intervening non-mass enhancement. The overall process occupies an area measuring about (12.3 x 12 x 9 cm) along its maximum dimensions, extending till about 3.5 cm from the nipple.

LEFT BREAST:

- Low energy (Non contrast) images: No masses, asymmetry or distortion or suspicious microcalcifications.
- Recombined (Post contrast) images show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION:

RIGHT BREAST: Upper outer quadrant and upper central (from 10 to 12:30 o'clock) pathological mass and non-mass enhancement with extension as above described (BIRADS 6)

LEFT BREAST: No pathological mass or non-mass enhancement (BIRADS 2= benign)

Best regards,

Dr. Sherifan Fakhry, MD
 Dr. Eman Ali, MSc

Patient Name : الأستاذ / د.عبد الحليم
Patient No. : 27701010127262
Path No. : 2196/25
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13003993
Request Date : 02/07/2025
Verification Date : 07/07/2025
Financial Category : نقابة الأطباء - Out Patient
Department :
Registered by : Mrs. Mayada Hegazy

Pathology Report

Right breast mass, Tru cut biopsy:
Mixed Invasive classic lobular & ductal carcinoma, Grade 2, with focal low grade Ductal
carcinoma in situ
IFT:
• ER: Positive (score 8/8).
• PR: Positive (score 8/8).
• HER2: Negative (score 0).
• Ki67: 26 %

Signature
Dr. Walea Ibrahim Ghoneim
Dr. Heba Hassan Adly

Verified by:
Dr. Heba Hassan

Printing Date : 14/07/2025 09:23:39

Page 2 of 2

Printing By : Mrs. Mayada Hegazy

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Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mizr El Qadeem - Cairo, Egypt

Patient Name: Doaa Khairi Abdel Haleem Mohamed		Date : 28/06/2025
Exam : ultrasound BREAST (BOTH SIDE)		Age : 48 Year(s)
Medical ID: 27701010127262		Pat. ID: 27596

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

- Breast density: Scattered areas of glandular tissue in both breasts... Matching
- ACR b.
- Multiple small irregular shaped masses are seen in the right upper outer quadrant.
- No detected definite suspicious mass density, or pathological micro-calcification in the left breast.
- Normal skin and nipple on both sides.

By complementary breast ultrasound:

- Homogenous (predominantly glandular) back ground parenchymal echo-texture.
- Multiple irregular shaped masses are seen scattered in the right breast in between 10 o'clock, zone 3 to 12-1 o'clock, zone 2, the largest is seen at 12 o'clock, zone 2 measuring (14 x 11 mm)
- No detected suspicious masses in the left breast.
- Bilateral nonspecific axillary lymph nodes.

CONCLUSION:

- Right breast : Multifocal highly suspicious masses (BI-RADS 4C category).
- Left breast : normal study : (BI-RADS 1 category).

MUCH OBLIGED

DR. MONA ZAKY, MD.

Professor of diagnostic radiology - Breast Imaging - Mansoura University

Signature Dr. Mona Zaki
28/06/2025 2:31 PM

Patient's Name:

Doaa Khairy Abdelhalim Mohammed

Date:

2026-09-09

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (15cm at Rt MCL) bright parenchymal echo pattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

Average size , homogenous.

• **Kidneys: -**

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Not distended enough however no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected, JUCD is in place

Opinion:

Bright liver likely fatty

Colonic gaseous distention

Dr/Nermeen ramadan.MD

Patient's Name: Doaa Kl im Mohammed
Date: 2026-09-09
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Status post right MRM

Findings:

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections.
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

Conclusion:-

- *Left breast (BIRADS 1): normal study.*
- *Clear right mastectomy bed and axilla.*

Recommendation:-

- *For annual follow up.*

Best Regards,

DR. Marwa M.Hassan

Name: DOAA KHAIRY ABDELHALEEM
Patient ID: 230820269
DOB: 01 January 1977

Sex: Female
Ethnicity: Hispanic

Height: 162.0 cm
Weight: 71.0 kg
Age: 49

Referring Physician:

Scan Information:

Scan Date: 23 August 2026

ID: A0823260U

Scan Type: f Lumbar Spine

Analysis: 23 August 2026 12:24 Version 13.6.1.3-5
Spine

Operator:

Model: Horizon Wi (S/N 306655M)

Comment:

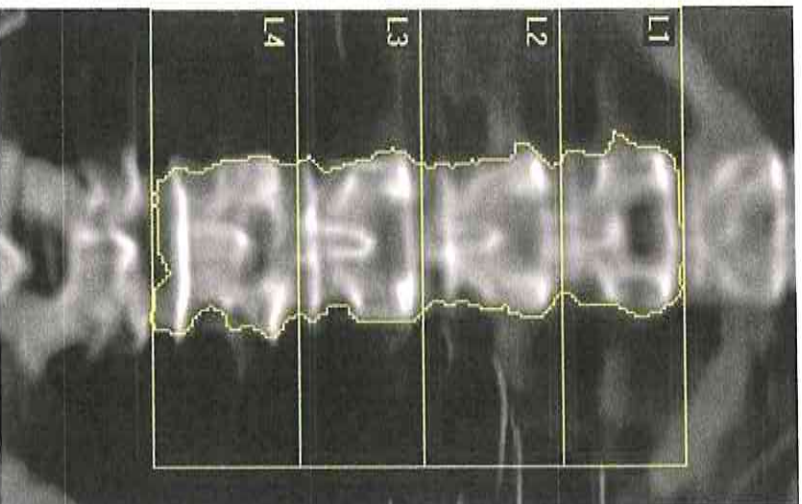
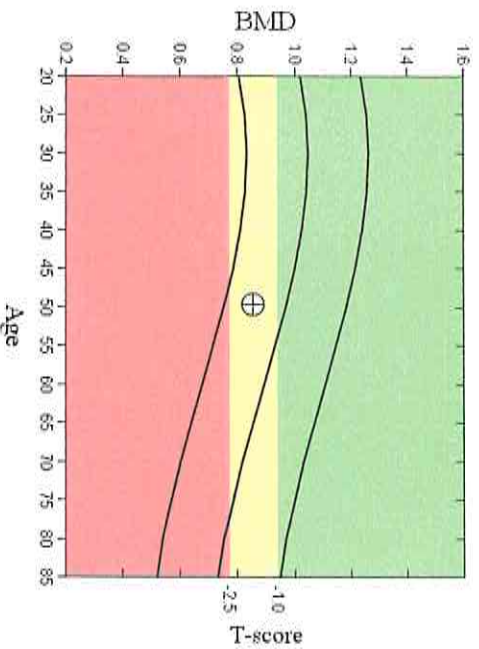


Image not for diagnostic use
k = 1.146, d0 = 46.9
116 x 135
DAP: 2.2 cGy*cm²

L1-L4



DXA Results Summary:

Region	Area (cm ²)	BMC (g)	BMD (g/cm ³)	T-score	PR (%)	Z-score	AM (%)
L1	12.30	9.59	0.780	-1.9	79	-1.3	85
L2	13.44	11.63	0.866	-1.5	84	-0.8	91
L3	12.59	11.36	0.903	-1.6	83	-0.9	90
L4	14.94	12.93	0.865	-1.8	82	-1.0	88
L1-L4	53.26	45.51	0.854	-1.8	82	-1.0	88
L2-L4	40.97	35.92	0.877	-1.8	81	-1.1	88

Total BMD CV 1.0%, ACF = 1.034, BCF = 1.013, TH = 7.126

WHO Classification: Osteopenia

Fracture Risk: Increased

Comment:

T-score vs. White Female. Source:2012 BMDCS/Hologic Z-score vs. Hispanic Female. Source:Hologic

HOLOG

Name: DOAA KHAIRY ABDELHALEEM
Patient ID: 230820269
DOB: 01 January 1977

Sex: Female
Ethnicity: Hispanic

Height: 162.0 cm
Weight: 71.0 kg
Age: 49

Referring Physician:

Scan Information:

Scan Date: 23 August 2026 ID: A0823260V
Scan Type: f Left Hip
Analysis: 23 August 2026 12:24 Version 13.6.1.3-5

Hip

Operator:
Model: Horizon Wi (S/N 306655M)

Comment:

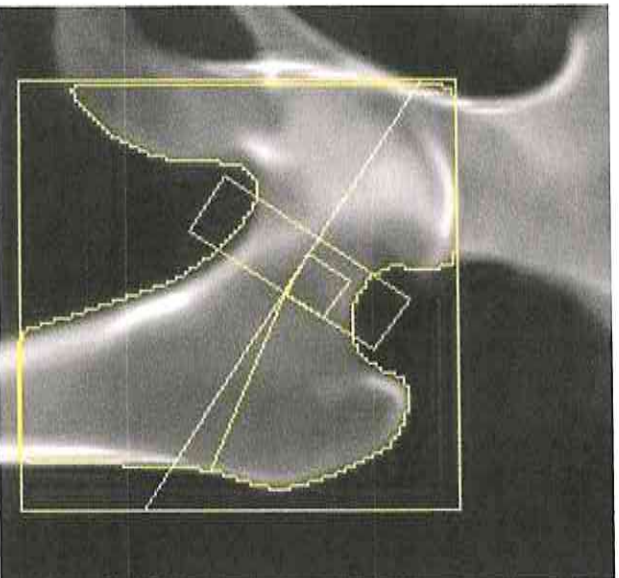


Image not for diagnostic use
 $k = 1.154$, $d0 = 51.4$
 95×97

NECK: 49×15
DAP: $1.5 \text{ cGy} \cdot \text{cm}^2$

DXA Results Summary:

Region	Area (cm^2)	BMC (g)	BMD (g/cm^2)	T - score	PR (%)	Z - score	AM (%)
Neck	4.48	3.33	0.744	-0.9	88	-0.5	93
Troch	11.71	7.13	0.609	-0.9	87	-0.5	92
Inter	18.35	17.47	0.952	-1.0	87	-0.8	88
Total	34.54	27.93	0.808	-1.1	86	-0.8	89
Wards	1.14	0.66	0.581	-1.3	79	-0.3	94

Total BMD CV 1.0%, ACF = 1.034, BCF = 1.013, TH = 5.974

WHO Classification: Osteopenia



10-year Fracture Risk¹

Major Osteoporotic Fracture

2.1%

Hip Fracture

0.1%

Reported Risk Factors:

US (Hispanic), Neck BMD=0.744, BMI=27.1

¹ FRAX® Version 3.08. Fracture probability calculated for an untreated patient. Fracture probability may be lower if the patient has received treatment.

Comment:

All treatment decisions require clinical judgment and consideration of individual patient factors, including patient preferences, comorbidities, previous drug use and risk factors not captured in the FRAX model (e.g. frailty, falls, vitamin D deficiency, increased bone turnover, interval significant decline in BMD).

T-score vs. White Female. Source:2012 BMDCS/NHANES White Female. Z-score vs. Hispanic Female. Source:NHANES

HOLOG

Name: DOAA KHAIRY ABDELHALEEM
Patient ID: 230820269
DOB: 01 January 1977

Sex: Female
Ethnicity: Hispanic

Height: 162.0 cm
Weight: 71.0 kg
Age: 49

Referring Physician:

Scan Information:

Scan Date: 23 August 2026 ID: A0823260X
Scan Type: a R.Forearm
Analysis: 23 August 2026 12:27 Version 13.6.1.3:5
Right Forearm

Operator:
Model: Horizon Wi (S/N 306655M)

Comment:

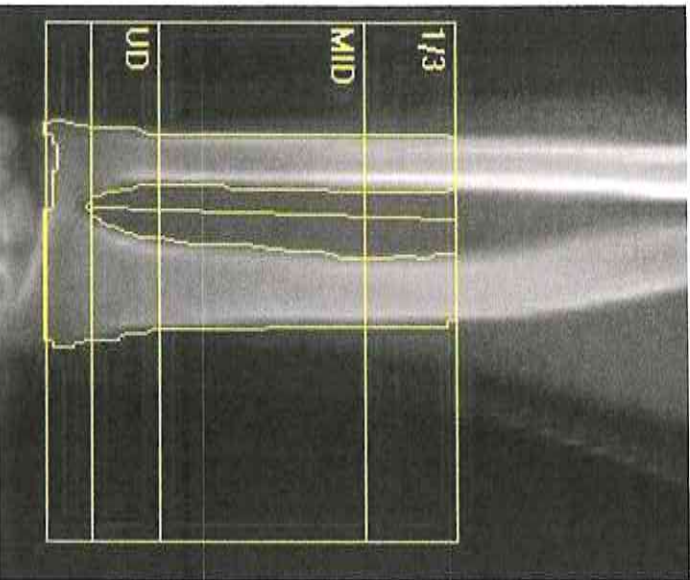
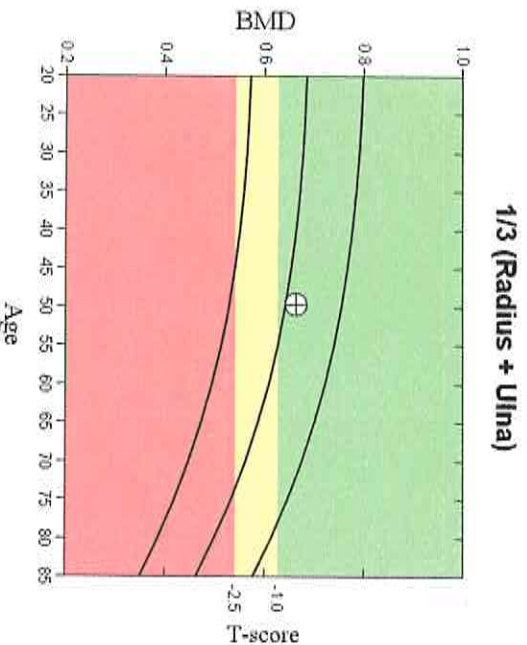


Image not for diagnostic use
k = 1.256, d0 = 71.3
228 x 91, Forearm Length: 24.0 cm
DAP: 1.6 cGy*cm²

DXA Results Summary:

Radius + Ulna	Area (cm²)	BMC (g)	BMD (g/cm³)	T - score	PR (%)	Z - score	AM (%)
1/3	4.94	3.28	0.663	-0.4	97	0.4	103
MID	10.93	6.01	0.550	-0.7	94	0.1	101
UD	4.95	2.08	0.420	0.2	102	0.7	109
Total	20.82	11.36	0.546	-0.4	97	0.4	104

Total BMD CV 1.0%, ACF = 1.034, BCF = 1.013
WHO Classification: Normal
Fracture Risk: Not Increased



Comment:

T-score vs. White Female. Source: Hologic Z-score vs. White Female. Source: Hologic